

Competitive Intelligence Briefing: Gracey Care

Gold-standard example — the quality bar the redesigned prompt targets (written on 24 real active ads)

Evidence base: 24 active ads → 13 unique creatives, collected from the Meta Ad Library. All quotes and counts are from the real ads. Spend, reach, and targeting are not public and are never stated.

1. Executive Summary

Gracey Care is a Medicare-billed chronic-care-coordination service running a lean, test-driven lead-gen account aimed not at seniors but at their adult children. The job the account is hired to do is singular: convert anxious family caregivers into "free, covered-by-Medicare" sign-ups. Their scaled winner is emotional, not functional — **"The Care Nobody Prepared You For"** runs across the most ad IDs for the longest stretch, beating their literal "Free Care Coordination" offer creative. The account's defining feature is a **geo-rotation** across four low-cost Mountain-West states (Arizona, Utah, Idaho, Wyoming) — and its defining vulnerability is that **not one of the 13 creatives contains a single piece of proof**: no patient name, no photo, no outcome, no nurse credential. In a category built on trusting a stranger with your parent's health, that is a wide-open flank.

2. Biggest Strategic Insight

They have already A/B-tested emotion vs. offer — and emotion won. Their three highest-conviction creatives ("The Care Nobody Prepared You For", "The Help Was Already There", "90% of Medicare Patients Don't Know This Exists") are all curiosity-and-guilt hooks. Their explicit-offer creative ("Free Care Coordination for Arizona Seniors") exists but is **not** what they've scaled. The tell: Gracey has learned that with caregivers, you sell the *feeling of having failed and being rescued* first, and the *free covered service* second. A competitor can skip months of testing by starting from that conclusion — then out-execute it with the proof Gracey lacks.

3. Company Positioning

Positioned as the quiet, local rescuer for families in over their heads — **"Arizona Families: You're Not Alone"**, **"Help For Families Managing Care"**. They deliberately cast no villain (unlike advocacy peers who attack insurers). The frame is relief, not fight: the help *"was already there"*, you simply didn't know. That softness is a positioning choice with a cost (see §5).

4. Messaging Strategy

Three message clusters, ranked by conviction (duplication × runtime):

1. **Guilt-and-rescue (scaled hardest):** "The Care Nobody Prepared You For" (4 ad IDs) / "The Help Was Already There" (3). Presses the caregiver's private fear of having failed, then relieves it.
2. **Hidden-entitlement curiosity:** "90% of Medicare Patients Don't Know This Exists" (3) / "Medicare Covers This — Most Families Don't Know" (2). Sells secret knowledge you're currently missing.
3. **Localized functional offer (run, not scaled):** "Care Coordination for Utah Seniors", "Free Care Coordination for Arizona Seniors". The literal what/where — used to localize, not to lead. The word doing the most work across the account is **"nobody / don't know"** — every top cluster runs on information asymmetry.

5. Customer Psychology

- **Anticipated regret + loss aversion (dominant emotion):** "The Care Nobody Prepared You For" — the "you" is the adult child, and the line implies you're already failing a parent unless you act. This, not fear of cost, is the account's engine.
- **Curiosity gap + inverted social proof:** "90% of Medicare Patients Don't Know This Exists" — everyone's missing it, so you're either the ignorant 90% or the smart 10% who clicks.
- **In-group identity as a trust shortcut:** naming the state — "Arizona Families", "Utah Seniors", "Most Wyoming Families" — makes a multi-state operation feel like a neighbor. It's the only trust device they use, and it's a cheap substitute for real proof. Emotion that dominates: **guilt relieved into belonging** — not the "savings relief" a cost-led competitor would assume.

6. Creative Winners

The scaled creatives share one DNA: an emotional or curiosity hook with the offer buried below. "The Care Nobody Prepared You For" (highest conviction) and "The Help Was Already There" both withhold what the service even is — they sell the click on feeling. What did NOT get scaled tells you as much: the explicit "Free Care Coordination for Arizona Seniors" carousel runs but sits low on conviction. Their testing verdict is clear — **lead with the wound, not the offer.**

7. Creative Breakdown

Predominantly single-image direct-response with two carousels that stack the offer ("Free Care Coordination... | ... | Medicare Covers This"). The carousels are the only place the literal offer leads — and they're low-conviction, reinforcing §6. There is no video, no UGC, no testimonial format anywhere. For a trust category, the absence of a single talking-head patient or nurse is conspicuous.

8. Hook Distribution

Concentrated, not sprayed: the guilt/curiosity family accounts for the large majority of scaled volume; the functional-offer hook trails. This is a **scaling** shape, not an **exploring** shape — Gracey has found its

message and is pushing it across geographies, changing the *state* in the copy far more often than the *hook*. Read: message-market fit reached; expansion, not discovery, is the current phase.

9. Offer Distribution

One offer, no variants: "free / covered by Medicare care coordination." No discount, no trial, no urgency, no scarcity anywhere in 13 creatives. That reveals the model — the service is Medicare-reimbursed (CCM), so there is no price to discount and discounting would raise compliance risk. The strategic consequence: their only conversion lever is emotional intensity, which is exactly why they lean so hard on guilt.

10. Funnel Strategy

100% conversion/lead-gen; zero brand or pure-education creative. Every ad is engineered to produce a qualified caregiver lead. They are harvesting existing intent, not building category awareness — sensible for a small operator, but it means they're wholly dependent on people who already sense they need help. A competitor who *creates* the realization (educates the not-yet-worried caregiver) reaches demand Gracey never touches.

11. Competitive Weaknesses

1. **Zero proof (critical):** across 13 creatives, no patient name, photo, testimonial, rating, "families served" count, or nurse/RN credential. In a hand-over-your-parent category this is the single largest hole.
2. **Abstraction:** "care coordination" is never made concrete — no "we call weekly, coordinate every doctor, catch problems before the ER." The prospect can't picture the value.
3. **No qualification:** CCM requires 2+ chronic conditions, but the ads never say so — likely inflating unqualified leads and cost per *good* lead.
4. **Soft frame, no enemy:** the relief framing under-activates vs. a named antagonist (fragmented care, doctors who don't talk to each other).
5. **Single-emotion dependence:** if guilt fatigue sets in, there's no second engine built.

12. Opportunities (the negative space)

What a sophisticated care-coordination advertiser would show that Gracey conspicuously omits: a **named care manager with a credential**, a **specific caught-it-in-time outcome**, a **concrete list of what the service does**, a **qualification gate**, and a **named enemy**. Each omission is an opening. The largest: proof. The prospect's unspoken question — "can I trust strangers with mom?" — goes completely unanswered in Gracey's ads.

13. Counter Strategy — if I launched a competitor tomorrow

1. **Attack the bare trust flank:** lead every ad with a named RN and a specific save — "Meet Diane, RN. In week one she caught a medication conflict my dad's three doctors missed." Gracey shows nothing like this; you own proof instantly.
2. **Out-specify the abstraction:** show the actual service as three concrete promises. Concreteness converts the anxious caregiver Gracey leaves guessing.
3. **Take the villain they won't:** name "the four-doctors-who-don't-talk problem." It activates harder than "the help was already there."
4. **Beat them in their own cheap markets first:** run proof+specificity in AZ/UT/ID/WY where they've already validated demand and CPMs are low — then expand into the CA/FL/TX markets they're avoiding, skipping their learning curve.
5. **Qualify in the ad** ("for seniors managing 2+ conditions") to undercut them on cost-per-*qualified*-lead where they run unqualified.

14. Action Items (this week)

1. Build 3 proof-led creatives (named RN + specific outcome) and run them head-to-head against Gracey's guilt hook in Arizona.
2. Write one "what we actually do" concrete-service ad to own the specificity gap.
3. Test a named-enemy hook ("The 4 doctors who never talk to each other") against Gracey's soft frame.
4. Launch in Utah and Idaho (their validated, low-cost markets) before contesting saturated states.
5. Add a 2+-chronic-conditions qualifier to every ad and measure cost-per-qualified-lead vs. an unqualified control.
6. Stand up a second emotional engine (pride/relief) so you're not, like Gracey, one guilt-hook deep.

This is the target quality: every claim cites a quoted line, a count, or a named geography; every section carries a non-obvious inference; the counter-strategy attacks specific, evidenced gaps. The redesigned prompt is engineered to produce this from the same data.